

Cohere Medical Policy – Shoulder Arthroscopy

Clinical Policy for Medical Necessity Review

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Procedure Codes (CPT/HCPCS)

Service: Shoulder Arthroscopy, Adjunctive/Soft Tissue Procedures

Code	Description
23929	Unlisted procedure, shoulder
29805	Arthroscopy, shoulder, diagnostic, with or without synovial biopsy (separate procedure)
29807	Arthroscopy, shoulder, surgical; repair of SLAP lesion
29819	Arthroscopy, shoulder, surgical; with removal of loose body or foreign body
29820	Arthroscopy, shoulder, surgical; synovectomy, partial
29821	Arthroscopy, shoulder, surgical; synovectomy, complete
29822	Arthroscopy, shoulder, surgical; debridement, limited, 1 or 2 discrete structures (eg, humeral bone, humeral articular cartilage, glenoid bone, glenoid articular cartilage, biceps tendon, biceps anchor complex, labrum, articular capsule, articular side of the rotator cuff, bursal side of the rotator cuff, subacromial bursa, foreign body[ies])
29823	Arthroscopy, shoulder, surgical; debridement, extensive, 3 or more discrete structures (eg, humeral bone, humeral articular cartilage, glenoid bone, glenoid articular cartilage, biceps tendon, biceps anchor complex, labrum, articular capsule, articular side of the rotator cuff, bursal side of the rotator cuff, subacromial bursa, foreign body[ies])
29824	Arthroscopy, shoulder, surgical; distal claviclectomy including distal articular surface (Mumford procedure)
29826	Arthroscopy, shoulder, surgical; decompression of subacromial space with partial acromioplasty, with coracoacromial ligament (ie, arch) release, when performed (List separately in addition to code for primary procedure)
29828	Arthroscopy, shoulder, surgical; biceps tenodesis
29999	Unlisted procedure, arthroscopy

Service: Shoulder Arthroscopy, Rotator Cuff Repair (RCR)

Code	Description
29827	Arthroscopy, shoulder, surgical; with rotator cuff repair
C9781	Arthroscopy, shoulder, surgical; with implantation of subacromial spacer (e.g., balloon), includes debridement (e.g., limited or extensive), subacromial decompression, acromioplasty, and biceps tenodesis when performed

Service: Shoulder Arthroscopy, Capsular and Diagnostic Procedures

Code	Description
23450	Capsulorrhaphy, anterior; Putti-Platt procedure or Magnuson type operation

23455	Capsulorrhaphy, anterior; with labral repair (eg, Bankart procedure)
23460	Capsulorrhaphy, anterior, any type; with bone block
23462	Capsulorrhaphy, anterior, any type; with coracoid process transfer
23465	Capsulorrhaphy, glenohumeral joint, posterior, with or without bone block
23466	Capsulorrhaphy, glenohumeral joint, any type multidirectional instability
23929	Unlisted procedure, shoulder
29805	Arthroscopy, shoulder, diagnostic, with or without synovial biopsy (separate procedure)
29806	Arthroscopy, shoulder, surgical; capsulorrhaphy
29825	Arthroscopy, shoulder, surgical; with lysis and resection of adhesions, with or without manipulation
S2300	Arthroscopy, shoulder, surgical; with thermally-induced capsulorrhaphy

Important Notices

Notices & Disclaimers:

GUIDELINES ARE SOLELY FOR COHERE'S USE IN PERFORMING MEDICAL NECESSITY REVIEWS AND ARE NOT INTENDED TO INFORM OR ALTER CLINICAL DECISION-MAKING OF END USERS.

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Service: Shoulder Arthroscopy - Description

A shoulder arthroscopy is a minimally invasive surgical technique that may involve one or more procedures, including the repair of shoulder components such as the rotator cuff, labrum, ligaments, or inflamed tissue or cartilage. Through a small incision and with the aid of an arthroscope, the surgeon can thoroughly visualize the shoulder and treat several conditions involving its structures.¹⁻⁹

Service: Shoulder Arthroscopy, Adjunctive/Soft Tissue Procedures - Medical Necessity Criteria

Administrative Guidelines for Medical Necessity Review

Cohere Health takes an evidence-based approach to reviewing imaging and procedure requests, meaning that sufficient clinical information must be provided at the time of submission to determine medical necessity. Documentation must include a recent and detailed history, physical examination related to the onset or change in symptoms, relevant lab results, prior imaging, and details of previous treatments. Advanced imaging or procedures should be requested after a recent clinical evaluation by the treating provider, which may include referral to a specialist.

- When a specific clinical indication is not explicitly addressed in the Cohere Health medical policy, medical necessity will be determined based on established clinical best practices, as supported by evidence-based literature, peer-reviewed sources, professional society guidelines, and state or national recommendations, unless otherwise directed by the health plan.
- Requests submitted without clinical documentation, or those that do not align with the provided clinical information—such as mismatched procedure, laterality, body part, or CPT code—may be denied for lack of medical necessity due to insufficient or inconsistent clinical information.
- When there are multiple diagnostic or therapeutic procedures requested simultaneously or within the past three months, each will be reviewed independently. Clinical documentation must clearly justify all of the following:
 - The medical necessity of each individual request
 - Why prior imaging or procedures were inconclusive, or why additional/follow-up studies are needed
 - How the results will impact patient management or treatment decisions
- Requests involving adjacent or contiguous body parts may be considered not medically necessary if the documentation demonstrates that the patient's primary symptoms can be adequately assessed with a single study or procedure.

Indications

- **Shoulder arthroscopy, adjunctive/soft tissue procedures** are considered appropriate if **ALL** of the following are **TRUE**:
 - Significant pain or functional impairment that impacts activities of daily living (ADLs)^{1,6}; **AND**
 - **ANY** of the following:
 - The procedure is performed as part of a medically necessary surgery⁶; **OR**
 - **ALL** of the following:
 - Failure of conservative management^A, including **ALL** of the following:
 - ▲ Anti-inflammatory medications, non-opioid analgesics, neuropathic medications, or other pain medications, if not contraindicated; **AND**
 - ▲ **ANY** of the following:
 - △ **ALL** of the following:
 - ◆ Physical therapy (PT) and/or documentation of patient compliance with a home exercise prescription/plan (HEP) provided by a physician or physical therapist targeting the requested region; **AND**
 - ◆ PT/HEP trialed for a duration of 3 months or more; **OR**
 - △ Progression or worsening of symptoms due to PT/HEP; **AND**
 - **ANY** of the following:
 - ▲ Corticosteroid injection if medically appropriate¹⁰; **OR**
 - ▲ Documentation that corticosteroid injection is contraindicated; **AND**

- **ANY** of the following:
 - Subacromial decompression and **ALL** of the following^{11,12}:
 - Clinical examination findings consistent with impingement syndrome¹³; **AND**
 - Imaging demonstrates the presence of type II/III or hooked acromion¹²; **OR**
 - Distal clavicle resection and **ALL** of the following^{6,11}:
 - Examination findings positive for acromioclavicular joint (ACJ) pain; **AND**
 - Imaging findings demonstrate **ANY** of the following¹⁴:
 - ▲ Inferior osteophytes impinging upon the rotator cuff; **OR**
 - ▲ Moderate to severe ACJ arthritis; **OR**
 - Biceps tenodesis or tenotomy and **ANY** of the following¹⁵:
 - Advanced diagnostic imaging demonstrates the presence of biceps pathology; **OR**
 - Examination findings positive for biceps pain; **OR**
 - The procedure is a debridement, and imaging studies demonstrate bony or soft tissue pathology that correlates with symptoms and physical exam findings; **OR**
 - Loose body removal and **ALL** of the following^{11,16}:
 - Imaging findings demonstrate the presence of a loose body; **AND**
 - Shoulder pain and mechanical symptoms; **OR**
 - Superior labrum anterior and posterior (SLAP) repair and **ALL** of the following^{11,17,18}:
 - Examination findings consistent with symptomatic SLAP tear; **AND**
 - Imaging findings demonstrate a superior labral tear consistent with exam findings; **OR**
 - Synovectomy and **ANY** of the following^{19,20}:
 - History or examination consistent with symptomatic synovitis; **OR**
 - Image findings demonstrate pathological synovium.

Non-Indications

- **Shoulder arthroscopy** is not considered appropriate if **ANY** of the following is **TRUE^B**:
 - Arthroscopic debridement or removal of intra-articular loose body is not indicated in the presence of Kellgren-Lawrence grade 3 or 4 osteoarthritis^{19,28,29}; **OR**
 - Biotuberplasty, biceps sling, or superior capsular reconstruction for irreparable rotator cuff tears³⁰⁻³²; **OR**
 - Rotator cuff arthropathy³³; **OR**
 - Rotator cuff or labral repair in the presence of active infection³⁴; **OR**
 - Thermal capsulorrhaphy³⁵; **OR**
 - Use of subacromial balloon spacer to treat an irreparable rotator cuff tear.^{36,37}

Definitions and Notes

^AFailure of conservative management:

- The patient has completed a full course of conservative management without meaningful improvement.
- Patient noncompliance, closure of therapy offices, and patient inconvenience do *not* constitute a failure of conservative treatment.

Service: Shoulder Arthroscopy, Rotator Cuff Repair (RCR) - Medical Necessity Criteria

Administrative Guidelines for Medical Necessity Review

Cohere Health takes an evidence-based approach to reviewing imaging and procedure requests, meaning that sufficient clinical information must be provided at the time of submission to determine medical necessity. Documentation must include a recent and detailed history, physical examination related to the onset or change in symptoms, relevant lab results, prior imaging, and details of previous treatments. Advanced imaging or procedures should be requested after a recent clinical evaluation by the treating provider, which may include referral to a specialist.

- When a specific clinical indication is not explicitly addressed in the Cohere Health medical policy, medical necessity will be determined based on established clinical best practices, as supported by evidence-based literature, peer-reviewed sources, professional society guidelines, and state or national recommendations, unless otherwise directed by the health plan.
- Requests submitted without clinical documentation, or those that do not align with the provided clinical information—such as mismatched procedure, laterality, body part, or CPT code—may be denied for lack of medical necessity due to insufficient or inconsistent clinical information.
- When there are multiple diagnostic or therapeutic procedures requested simultaneously or within the past three months, each will be reviewed independently. Clinical documentation must clearly justify all of the following:
 - The medical necessity of each individual request
 - Why prior imaging or procedures were inconclusive, or why additional/follow-up studies are needed
 - How the results will impact patient management or treatment decisions
- Requests involving adjacent or contiguous body parts may be considered not medically necessary if the documentation demonstrates that the patient's primary symptoms can be adequately assessed with a single study or procedure.

Indications

- **Shoulder arthroscopy, rotator cuff repair (RCR)** is considered appropriate if **ANY** of the following is **TRUE**^{4,6,11,21}.
 - **ALL** of the following:
 - Advanced imaging with radiologist report that demonstrates **ANY** of the following:
 - Full-thickness rotator cuff tear; **OR**
 - High-grade partial-thickness rotator cuff tear; **AND**
 - Significant pain or functional impairment that impacts activities of daily living (ADLs); **AND**
 - **ANY** of the following:
 - Documentation of an acute rotator cuff tear (RCT) after an inciting event or injury; **OR**
 - Documentation of a chronic RCT and **ALL** of the following²²:
 - ▲ Failure of conservative management^A, including **ALL** of the following:
 - △ Anti-inflammatory medications, non-opioid analgesics, neuropathic medications, or other pain medications, if not contraindicated; **AND**
 - △ **ANY** of the following:
 - ◆ **ALL** of the following:
 - ◇ Physical therapy (PT) and/or documentation of patient compliance with a home exercise prescription/plan (HEP) provided by a physician or physical therapist targeting the requested region; **AND**
 - ◇ PT/HEP trialed for a duration of 3 months or more; **OR**

◆ Progression or worsening of symptoms due to PT/HEP; **AND**

▲ **ANY** of the following:

△ Corticosteroid injection if medically appropriate¹⁰; **OR**

△ Documentation that corticosteroid injection is contraindicated; **OR**

○ A revision of a previous rotator cuff repair and **ALL** of the following^{17,23}:

■ Significant pain or functional impairment that impacts ADLs; **AND**

■ **ANY** of the following:

□ Advanced diagnostic imaging findings document a recurrent rotator cuff tear; **OR**

□ Suspected postsurgical complication.

Non-Indications

● **Shoulder arthroscopy** is not considered appropriate if **ANY** of the following is **TRUE**^B:

○ Arthroscopic debridement or removal of intra-articular loose body is not indicated in the presence of Kellgren-Lawrence grade 3 or 4 osteoarthritis^{19,28,29}; **OR**

○ Biotuberplasty, biceps sling, or superior capsular reconstruction for irreparable rotator cuff tears³⁰⁻³²; **OR**

○ Rotator cuff arthropathy³³; **OR**

○ Rotator cuff or labral repair in the presence of active infection³⁴; **OR**

○ Thermal capsulorrhaphy³⁵; **OR**

○ Use of subacromial balloon spacer to treat an irreparable rotator cuff tear.^{36,37}

Definitions and Notes

^A**Failure of conservative management:**

● The patient has completed a full course of conservative management without meaningful improvement.

● Patient noncompliance, closure of therapy offices, and patient inconvenience do *not* constitute a failure of conservative treatment.

Service: Shoulder Arthroscopy, Capsular and Diagnostic Procedures - Medical Necessity Criteria

Administrative Guidelines for Medical Necessity Review

Cohere Health takes an evidence-based approach to reviewing imaging and procedure requests, meaning that sufficient clinical information must be provided at the time of submission to determine medical necessity. Documentation must include a recent and detailed history, physical examination related to the onset or change in symptoms, relevant lab results, prior imaging, and details of previous treatments. Advanced imaging or procedures should be requested after a recent clinical evaluation by the treating provider, which may include referral to a specialist.

- When a specific clinical indication is not explicitly addressed in the Cohere Health medical policy, medical necessity will be determined based on established clinical best practices, as supported by evidence-based literature, peer-reviewed sources, professional society guidelines, and state or national recommendations, unless otherwise directed by the health plan.
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 - The medical necessity of each individual request
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 - How the results will impact patient management or treatment decisions
- Requests involving adjacent or contiguous body parts may be considered not medically necessary if the documentation demonstrates that the patient's primary symptoms can be adequately assessed with a single study or procedure.

Indications

- **Shoulder arthroscopy, capsular and diagnostic procedures** are considered appropriate if **ANY** of the following is **TRUE**:
 - A capsular release and **ALL** of the following^{7-9,11,24}:
 - Failure of conservative management^A, including **ALL** of the following:
 - Anti-inflammatory medications, non-opioid analgesics, neuropathic medications, or other pain medications, if not contraindicated; **AND**
 - **ANY** of the following:
 - ▲ **ALL** of the following:
 - △ Physical therapy (PT) and/or documentation of patient compliance with a home exercise prescription/plan (HEP) provided by a physician or physical therapist targeting the requested region; **AND**
 - △ PT/HEP trialed for a duration of 3 months or more; **OR**
 - ▲ Progression or worsening of symptoms due to PT/HEP; **AND**
 - **ANY** of the following:
 - ▲ Corticosteroid injection if medically appropriate¹⁰; **OR**
 - ▲ Documentation that corticosteroid injection is contraindicated; **AND**
 - Imaging findings do not identify any other shoulder pathology as the primary source of the symptoms; **AND**
 - Significant loss of both active and passive shoulder range of motion on examination; **AND**

- Significant pain or functional impairment that impacts activities of daily living (ADLs); **OR**
- A capsulorrhaphy/labral repair for shoulder instability and **ALL** of the following^{11,23-25}:
 - Advanced diagnostic imaging consistent with instability; **AND**
 - Documented history of traumatic or atraumatic shoulder laxity; **AND**
 - Documented loss of shoulder function that impacts activities of daily living or demands of employment; **AND**
 - Physical examination consistent with instability; **AND**
 - **ANY** of the following:
 - Acute traumatic injury less than or equal to 30 days old; **OR**
 - Failure of conservative management^A, including **ALL** of the following:
 - ▲ Anti-inflammatory medications, non-opioid analgesics, neuropathic medications, or other pain medications, if not contraindicated; **AND**
 - ▲ **ANY** of the following:
 - △ **ALL** of the following:
 - ◆ Physical therapy (PT) and/or documentation of patient compliance with a home exercise prescription/plan (HEP) provided by a physician or physical therapist targeting the requested region; **AND**
 - ◆ PT/HEP trialed for a duration of 3 months or more; **OR**
 - △ Progression or worsening of symptoms due to PT/HEP; **AND**
 - ▲ **ANY** of the following:
 - △ Corticosteroid injection if medically appropriate¹⁰; **OR**
 - △ Documentation that corticosteroid injection is contraindicated; **OR**
- The procedure is a diagnostic arthroscopy and **ALL** of the following^{9,11,19,23}:
 - Significant pain or functional impairment that impacts activities of daily living; **AND**
 - **ANY** of the following positive exam findings:
 - Decreased range of motion; **OR**
 - Instability; **OR**
 - Painful shoulder range of motion; **OR**
 - Weakness; **AND**
 - **ANY** of the following:
 - Imaging findings inconclusive as to the source of shoulder pain; **OR**
 - Synovial biopsy is required for definitive diagnosis^{26,27}; **AND**
 - Failure of conservative management^A, including **ALL** of the following:
 - Anti-inflammatory medications, non-opioid analgesics, neuropathic medications, or other pain medications, if not contraindicated; **AND**
 - **ANY** of the following:
 - ▲ **ALL** of the following:
 - △ Physical therapy (PT) and/or documentation of patient compliance with a home exercise prescription/plan (HEP) provided by a physician or physical therapist targeting the requested region; **AND**
 - △ PT/HEP trialed for a duration of 3 months or more; **OR**
 - ▲ Progression or worsening of symptoms due to PT/HEP; **AND**
 - **ANY** of the following:
 - ▲ Corticosteroid injection if medically appropriate¹⁰; **OR**
 - ▲ Documentation that corticosteroid injection is contraindicated.

Non-Indications

- **Shoulder arthroscopy** is not considered appropriate if **ANY** of the following is **TRUE**^B:
 - Arthroscopic debridement or removal of intra-articular loose body is not indicated in the presence of Kellgren-Lawrence grade 3 or 4 osteoarthritis^{19,28,29}; **OR**
 - Biotuberplasty, biceps sling, or superior capsular reconstruction for irreparable rotator cuff tears³⁰⁻³²; **OR**
 - Rotator cuff arthropathy³³; **OR**
 - Rotator cuff or labral repair in the presence of active infection³⁴; **OR**
 - Thermal capsulorrhaphy³⁵; **OR**
 - Use of subacromial balloon spacer to treat an irreparable rotator cuff tear.^{36,37}

Definitions and Notes

^A**Failure of conservative management:**

- The patient has completed a full course of conservative management without meaningful improvement.
- Patient noncompliance, closure of therapy offices, and patient inconvenience do *not* constitute a failure of conservative treatment.

Medical Evidence

Antonacci et al. (2025) conducted a systematic review to evaluate the changes in shoulder range of motion (ROM) after arthroscopic rotator cuff surgery. In the studies included in this systematic review, ROM was typically assessed through flexion, abduction, and internal and external rotation. Post-surgical ranges of motion, the study found, improved gradually, accelerating at 6 months, and peaking at 12 months. These results, the authors caution, were gleaned from ROM studies with variable patient characteristics and differing monitoring systems, and an absence of a standardized post-surgical protocol.³⁸

Kim et al. (2021) examined the predominance of arthroscopic distal clavicle excision procedures, specifically those using a fluoroscopic Kirchner wire guide. The technical aspects of the procedure were described, and the group concluded that the use of a fluoroscopic wire leads to surgical success, particularly in new orthopedic surgeons, as the distal clavicle excision is a technically demanding procedure due to visualization difficulties. The authors note that open distal clavicle excision remains the gold standard procedure for acromioclavicular joint arthritis.¹⁴

Redler et al. (2019) examined the literature on treating adhesive capsulitis of the shoulder and discussed the higher incidence of the diagnosis in diabetic patients, particularly those with long-standing, poorly controlled disease. Hyperthyroidism was determined in studies to be an independent risk factor for developing adhesive capsulitis. The authors preferred surgical technique post-nonsurgical interventions, including both anterior and posterior capsular release with rotator interval release and release of the coracohumeral ligament.⁸

Ansok et al. (2018) reviewed optimal management strategies for glenohumeral osteoarthritis, concluding that initial conservative treatment includes the use of viscosupplementation or corticosteroid injections. The age and function of the affected patient should determine the necessity of operative treatment, and operative treatment of young individuals with glenohumeral osteoarthritis remains controversial. Younger, more active patients do benefit from non-arthroplasty techniques or procedures with minimal bone resection.³⁹

Millett et al. (2018) examined the options for treating young, active patients with primary glenohumeral osteoarthritis, concluding that arthroplasty may not be ideal in that population. The authors note that clinical studies report arthroscopic approaches for procedures such as synovectomy, chondroplasty, loose body removal, capsular release, and biceps tenodesis may improve clinical outcomes. The authors conclude that arthroscopic management of glenohumeral osteoarthritis has numerous advantages over total shoulder arthroplasty; however, additional larger studies are needed.¹⁹

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Policy Revision History/Information

Clinical Policy Version	Date	Review History
Version 1	03/05/2021	Original Date.
Version 2	10/13/2023	Policy criteria reviewed and updated per medical literature.
Version 3	12/29/2023	Policy criteria reviewed and updated per medical literature.
Version 4	02/26/2024	Policy criteria reviewed and updated per medical literature.
Version 5	05/29/2025	Annual review. Corrected Errors in references/added new references. Added codes 23450-23466 (Capsulorrhaphy). Reorganized criteria. Specified that thermal capsulorrhaphy is non-indicated. Added non-indication for Biotuberplasty, biceps sling, and superior capsular reconstruction for irreparable rotator cuff tears.
Version 6	06/04/2026	Annual review. Rearranged bullets for improved usability and organization. Traumatic instability extended to less than or equal to 60 days old. Radiologist report requirement added for rotator cuff repair. Updated conservative care language. Expanded Medical Evidence section. Added 2 references.